

There are insufficient data for the withdrawal of concomitant antiepileptic medicinal products, once seizure control with pregabalin in the add-on situation has been reached, in order to reach monotherapy on pregabalin.

Tumorigenic potential

An unexpectedly high incidence of hemangiosarcoma was identified in two different strains of mice. The clinical significance of this finding is unknown. Clinical experience during pregabalin's premarketing development provides no direct means to assess its potential for inducing tumors in humans.

New or worsening pre-existing tumors were reported in patients greater than 12 years of age. Without knowledge of the background incidence and recurrence in similar populations not treated with pregabalin, it is impossible to know whether the incidence seen in these cohorts is or is not affected by treatment.

Ophthalmological effects

Although the clinical significance of the ophthalmologic findings is unknown, inform patients to notify their physician if changes in vision occur. If visual disturbance persists, consider further assessment. Consider more frequent assessment for patients who are already routinely monitored for ocular conditions.

In the postmarketing experience, visual adverse reactions have also been reported, including loss of vision, visual blurring or other changes of visual acuity, many of which were transient. Discontinuation of pregabalin may result in resolution or improvement of these visual symptoms.

Creatine kinase elevations

Pregabalin treatment was associated with creatine kinase elevations. Cases of rhabdomyolysis have been reported. The relationship between these myopathy events and pregabalin is not completely understood because the cases had documented factors that may have caused or contributed to these events. Instruct patients to promptly report unexplained muscle pain, tenderness, or weakness, particularly if these muscle symptoms are accompanied by malaise or fever. Discontinue treatment with pregabalin if myopathy is diagnosed or suspected or if markedly elevated creatine kinase levels occur.

Decreased platelet count

Pregabalin treatment was associated with a decrease in platelet count. A single pregabalin treated patient developed severe thrombocytopenia with a platelet count less than $20 \times 10^9/\text{microliter}$. Pregabalin was not associated with an increase in bleeding-related adverse reactions.

PR interval prolongation

Pregabalin treatment was associated with PR interval prolongation. In analyses of ECG data in adult patients, the mean PR interval increase was 3–6 msec at pregabalin doses greater than or equal to 300 mg/day. This dose class difference was associated with an increased risk of PR increase greater than or equal to 25% from baseline, an increased percentage of patients with on-treatment PR greater than 200 msec, or an increased risk of adverse reactions of second- or third-degree AV block.

Subgroup analyses did not identify an increased risk of PR prolongation in patients with baseline PR prolongation or in patients taking other PR prolonging medications. However, these analyses cannot be considered definitive because of the limited number of patients in these categories.

Women of childbearing potential/contraception

Pregabalin use in the first trimester of pregnancy may cause major birth defects in the unborn child. Pregabalin should not be used during pregnancy unless the benefit to the mother clearly outweighs the potential risk to the fetus. Women of childbearing potential must use effective contraception during treatment.

Others

Although the effects of discontinuation on the reversibility of renal failure have not been systematically studied, improved renal failure following discontinuation or dose reduction of pregabalin has been reported.

Misuse, abuse potential, or dependence

Cases of misuse, abuse, and dependence have been reported. Caution should be exercised in patients with a history of substance abuse and the patient should be monitored for symptoms of pregabalin misuse, abuse, or dependence (development of tolerance, dose escalation, drug seeking behavior have been reported).

Encephalopathy

Cases of encephalopathy have been reported, mostly in patients with underlying conditions that may precipitate encephalopathy.

Lactose intolerance

Pregabalin contains lactose monohydrate. Patients with rare hereditary problems of galactose intolerance, the Lapp-lactase deficiency or glucose-galactose malabsorption should not take this medicinal product.

Treatment of central neuropathic pain due to spinal cord injury

In the treatment of central neuropathic pain due to spinal cord injury the incidence of adverse reactions in general, central nervous system adverse reactions and especially somnolence was increased. This may be attributed to an additive effect due to concomitant medication (e.g., antispasmodic agents) needed for this condition. This should be considered when prescribing pregabalin in this condition.

Effects on ability to drive and use machines

Pregabalin may cause dizziness and somnolence and therefore may influence the ability to drive or use machines. Patients are advised not to drive, operate complex machinery, or engage in other potentially hazardous activities until it is known whether this medication affects their ability to perform these activities.

Interactions with Other Medicaments:

Since pregabalin is predominantly excreted unchanged in the urine, undergoes negligible metabolism in humans (<2% of a dose recovered in urine as metabolites), does not inhibit drug metabolism, and is not bound to plasma proteins, it is unlikely to produce, or be subject to, pharmacokinetic interactions.

Accordingly, no clinically relevant pharmacokinetic interactions were observed between pregabalin and phenytoin, carbamazepine, valproic acid, lamotrigine, gabapentin, lorazepam, oxycodone, or ethanol. Oral antidiabetics, diuretics, insulin, phenobarbital, tagabine, and topiramate had no clinically significant effect on pregabalin clearance.

Coadministration of pregabalin with the oral contraceptives norethisterone and/or ethinylestradiol does not influence the steady-state pharmacokinetics of either substance. Pregabalin may potentiate the effects of ethanol and lorazepam. Multiple oral doses of pregabalin coadministered with oxycodone, lorazepam, or ethanol did not result in clinically important effects on respiration. Pregabalin appears to be additive in the impairment of cognitive and gross motor function caused by oxycodone.

In the postmarketing experience, there are reports of respiratory failure, coma, and deaths in patients taking pregabalin and other CNS depressant medications including in patients who are substance abusers. There are postmarketing reports of events related to reduced lower gastrointestinal tract function (e.g., intestinal obstruction, paralytic ileus, constipation) when pregabalin was coadministered with medications that have the potential to produce constipation, such as opioid analgesics.

No specific pharmacodynamic interaction studies were conducted in elderly.

Pregnancy and Lactation:

Pregnancy

There is a limited amount of data on the use of pregabalin in pregnant women. Pregabalin should not be used during pregnancy unless the benefit to the mother clearly outweighs the potential risk to the fetus. Effective contraception must be used in women of childbearing potential.

Lactation

Pregabalin is excreted in the milk of lactating women (see **Pharmacokinetics**). As the safety of pregabalin in infants is not known, breastfeeding is not recommended during treatment with pregabalin. A decision must be made whether to discontinue breastfeeding or to discontinue from pregabalin therapy taking into account the benefit of breastfeeding for the child and the benefit of therapy for the woman.

Adverse Effects:

The most commonly reported adverse reactions were dizziness and somnolence. Adverse reactions were usually mild to moderate in intensity. The most common adverse reactions resulting in discontinuation from pregabalin treatment groups were dizziness and somnolence.

Selected adverse drug reactions that were treatment related are listed in the table below by system organ class (SOC). The frequency of these terms has been based on all-causality adverse drug reactions (very common ($\geq 1/10$), common ($\geq 1/100$, <1/10), uncommon ($\geq 1/1,000$, <1/100), and rare (<1/1,000)).

The adverse reactions listed may also be associated with the underlying disease and/or concomitant medications.

System organ class	Adverse drug reactions
Infections and infestations	
Common	Nasopharyngitis
Blood and lymphatic system disorders	
Uncommon	Neutropenia
Metabolism and nutrition disorders	
Common	Appetite increased
Uncommon	Anorexia, hypoglycemia
Psychiatric disorders	
Common	Euphoric mood, confusion, irritability, depression, disorientation, insomnia, libido decreased

Uncommon	Hallucination, restlessness, agitation, depressed mood, elevated mood, mood swings, depersonalization, abnormal dreams, word finding difficulty, libido increased, anorgasmia
Rare	
Nervous system disorders	
Very common	Dizziness, somnolence
Common	Ataxia, coordination abnormal, tremor, dysarthria, amnesia, memory impairment, disturbance in attention, paresthesia, hypoesthesia, sedation, balance disorder, lethargy
Uncommon	Syncope, myoclonus, psychomotor hyperactivity, dyskinesia, dizziness postural, intention tremor, nystagmus, cognitive disorder, speech disorder, hyporeflexia, hyperesthesia, burning sensation
Rare	Stupor, parosmia, hypokinesia, ageusia, dysgraphia
Eye disorders	
Common	Vision blurred, diplopia
Uncommon	Peripheral vision loss, visual disturbance, eye swelling, visual field defect, visual acuity reduced, eye pain, asthenopia, photopsia, dry eye, lacrimation increased, eye irritation
Rare	Oscillopsia, altered visual depth perception, mydriasis, strabismus, visual brightness
Ear and labyrinth disorders	
Common	Vertigo
Uncommon	Hyperacusis
Cardiac disorders	
Uncommon	Tachycardia, atrioventricular block first degree, sinus bradycardia
Rare	Sinus tachycardia, sinus arrhythmia
Vascular disorders	
Uncommon	Hypotension, hypertension, hot flushes, flushing, peripheral coldness
Respiratory, thoracic, and mediastinal disorders	
Uncommon	Dyspnea, epistaxis, cough, nasal congestion, rhinitis, snoring
Rare	Throat tightness, nasal dryness
Gastrointestinal disorders	
Common	Vomiting, constipation, flatulence, abdominal distension, dry mouth
Uncommon	Gastroesophageal reflux disease, salivary hypersecretion, hypoesthesia oral
Rare	Ascites, pancreatitis, dysphagia
Skin and subcutaneous tissue disorders	
Uncommon	Rash papular, urticaria, sweating
Rare	Cold sweat, Stevens-Johnson syndrome
Musculoskeletal and connective tissue disorders	
Common	Muscle cramp, arthralgia, back pain, pain in limb, cervical spasm
Uncommon	Joint swelling, myalgia, muscle twitching, neck pain, muscle stiffness
Rare	Rhabdomyolysis
Renal and urinary disorders	
Uncommon	Urinary incontinence, dysuria
Rare	Renal failure, oliguria
Reproductive system and breast disorders	
Uncommon	Erectile dysfunction, sexual dysfunction, ejaculation delayed, dysmenorrhea
Rare	Breast pain, amenorrhea, breast discharge, breast enlargement
General disorders and administration site conditions	
Common	Edema peripheral, edema, gait abnormal, fall, feeling drunk, feeling abnormal, fatigue
Uncommon	Generalized edema, chest tightness, pain, pyrexia, thirst, chills, asthenia
Investigations	
Common	Weight increased
Uncommon	Blood creatine phosphokinase increased, alanine aminotransferase increased, aspartate aminotransferase increased, blood glucose increased, platelet count decreased, blood potassium decreased, weight decreased
Rare	White blood cell count decreased, blood creatinine increased

The following adverse drug reactions were reported during postmarketing:

- Immune system disorder
- Uncommon: hypersensitivity
- Rare: angioedema, allergic reaction.
- Nervous system disorders
- Very common: headache
- Uncommon: loss of consciousness, mental impairment.
- Eye disorders
- Rare: keratitis
- Cardiac disorders
- Rare: congestive heart failure.
- Respiratory, thoracic, and mediastinal disorders
- Rare: pulmonary oedema.
- Gastrointestinal disorders
- Common: nausea, diarrhea.
- Rare: swollen tongue.
- Skin and subcutaneous tissue disorders
- Uncommon: face swelling, pruritus.
- Rare: Stevens-Johnson syndrome.
- Renal and urinary disorders
- Rare: urinary retention.
- Reproductive system and breast disorders
- Rare: gynecomastia.
- General disorders and administration site conditions
- Uncommon: malaise.

Overdose and Treatment:

In overdoses up to 15 g, no unexpected adverse reactions were reported. The most commonly reported adverse reactions observed when pregabalin was taken in overdose included affective disorder, somnolence, confusional state, depression, agitation, and restlessness. Seizures were also reported. Treatment of pregabalin overdose should include general supportive measures and may include hemodialysis if necessary.

Presentation and Registration Number:

Box, 3 blisters x 10 capsules; MALXXXXXXX

ON MEDICAL PRESCRIPTION ONLY.

STORE AT TEMPERATURES BELOW 30°C.

KEEP MEDICINE OUT OF REACH OF CHILDREN.

Manufactured by
PT Dexa Medica
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Palembang-Indonesia

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